

Table 1: Staffing Response to Ownership Change: Near-Capacity vs. Slack-Capacity Acquisitions

	Outcomes			
	RN	LPN	CNA	Total
Panel A: Staffing Levels (HPRD)				
Slack-capacity: <i>post</i>	−0.029*** (0.008)	−0.020** (0.010)	−0.063*** (0.015)	−0.112*** (0.021)
Difference (Near − Slack): <i>post</i> × Near-capacity	0.014 (0.010)	−0.004 (0.011)	0.005 (0.017)	0.015 (0.023)
Near-capacity effect: <i>post</i> + interaction	−0.015* (0.008)	−0.024*** (0.009)	−0.058*** (0.013)	−0.097*** (0.019)
Panel B: Log Staffing Levels				
Slack-capacity: <i>post</i>	−0.078*** (0.023)	−0.003 (0.014)	−0.031*** (0.008)	−0.034*** (0.006)
Difference (Near − Slack): <i>post</i> × Near-capacity	0.040 (0.031)	−0.012 (0.017)	0.004 (0.009)	0.004 (0.007)
Near-capacity effect: <i>post</i> + interaction	−0.038 (0.026)	−0.015 (0.013)	−0.027*** (0.007)	−0.030*** (0.006)

Notes: Sample restricted to ever-treated (CHOW) facilities only, split at the median pre-acquisition spare capacity (facility-level average spare capacity over $\text{event_time} \in [-12, -4]$, i.e., 4–12 months before ownership change, prior to the anticipation window).

Near-capacity = pre-acquisition average spare capacity ≤ 0.261 (median); Slack-capacity = above 0.261. $N = 1443$ treated facilities with a usable baseline (722 near-capacity, 721 slack-capacity).

All specifications include facility and calendar-month fixed effects and the standard control set (beds, government, non-profit, chain, occupancy rate, Medicare share, Medicaid share, and case-mix quartile indicators where available). The anticipation window is excluded.

Standard errors are clustered two ways by facility and calendar month. Statistical significance: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.

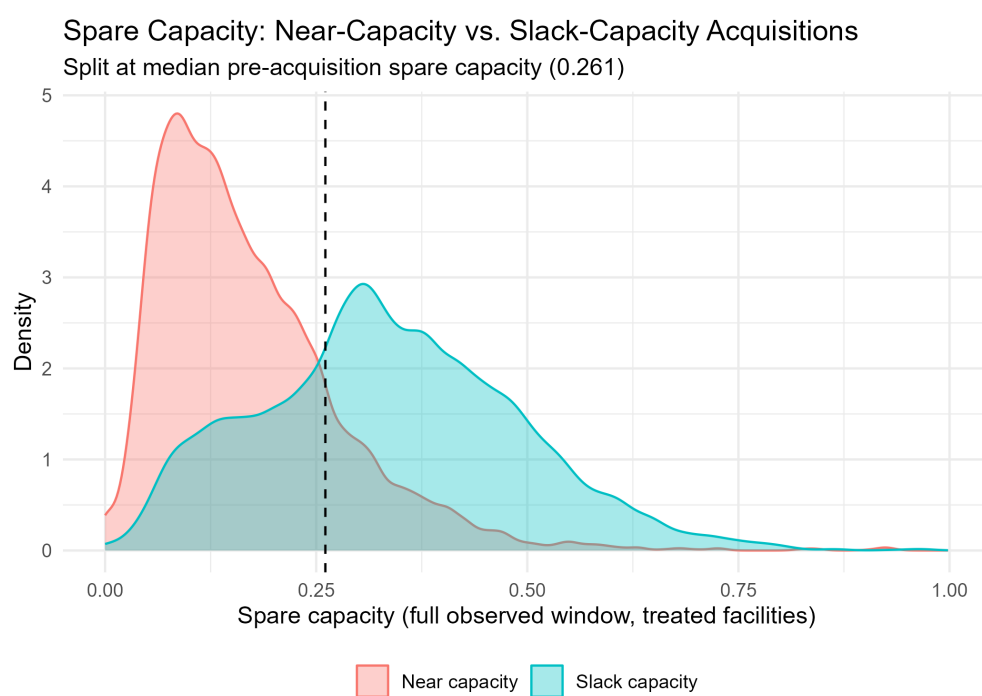


Figure 1: Spare Capacity Distribution by Acquisition Group